

Baseline blood pressure distribution, annual event probabilities and efficacy model inputs

Table 1. Baseline blood pressure distribution, annual event probabilities and efficacy model inputs

Parameter	Value	Source
Baseline distribution of systolic blood pressure values (mmHg)		
<120	2.6%	[18]
120-129	5.4%	[18]
130-139	10.1%	[18]
140-149	16.8%	[18]
150-159	17.0%	[18]
160-169	20.8%	[18]
≥170	27.3%	[18]
Annual probabilities for suffering a CV event for each systolic blood pressure category (mmHg)		
<120	0.00888712	[19]
120-129	0.00631655	[19]
130-139	0.00684717	[19]
140-149	0.00973564	[19]
150-159	0.00973564	[19]
160-169	0.00973564	[19]
≥170	0.00973564	[19]
Annual probabilities for CV disease related death		
<120	0.0035935	[18]
120-129	0.0029955	[18]
130-139	0.0029955	[18]
140-149	0.0033942	[18]
150-159	0.0033942	[18]
160-169	0.0051865	[18]
≥170	0.0051865	[18]
Annual probabilities for all-cause death (excluding CV disease related death)		
Age-based annual mortality	CBS	[25]
Annual probabilities for recurrent event		
Recurrent stroke (infarction/hemorrhage)	0.01158574	[26]

Recurrent MI	0.02539163	[26]
Recurrent event (composite)	0.01846044	[26]
HR post-event CV disease related death	3	[26]
Efficacy model inputs		
Annual reduction in systolic blood pressure due to drug therapy	-5.1 mmHg	[23]
Annual reduction in systolic blood pressure due to HBPT, 1 year ^A	-12 mmHg	[8]
Annual % of patients with resistant hypertension	19.7%	[24]

Abbreviations: CV = cardiovascular, MI = myocardial infarction. ^ASince most of the HBPT trials have follow-up durations up to 1 year, we assumed that HBPT would only cause an additional blood pressure lowering effect (in addition to the effect of drug therapy) in the first year (cycle 1).